



Discharge Summary

Name of the Patient	Jain Sangita	Payer	
Age	37	Date of Registration	31/03/2026
Gender	Female	Date of Admission	02/06/2026
UHID	KABHK.0000276322	Date of Discharge	03/06/2026
IPD number	BHKIP183545	Date of Treatment	Click or tap to enter a date.
Admitting Doctor	Dr. Shekar Patil	Type of Discharge	Doctors Advice
Co-managed Doctor	Dr. Shekar Patil		

Department of Medical Oncology

Diagnosis: Carcinoma left breast

ER Positive, PR Positive, Her2/Neu Negative, Ki-67 – 60%, clinically T2N1M0

Was patient treated in HCG for the Disease: Yes

Current Admission: For 4th cycle of neoadjuvant chemotherapy with AC regimen (3 weekly)

Brief History: Mrs. Jain Sangita, 37-year-old female, had complaints of lump in the left breast since 20 days, underwent evaluation with sonomammogram on 21.03.2026 suggestive of irregular hypoechoic lobulated lesion in the left breast, IVB suspicious of neoplastic with left axillary lymphadenopathy followed by further patient underwent FDG PET/CT scan on 24.03.2026 suggestive of FDG uptake seen in an ill defined mass lesion measuring approximately 1.8x1.4x2.2cm in the upper central quadrant minimal adjacent architecture distortion is noted. There is no evidence of infiltration of the angle and pectoralis with skin involvement anteriorly. Deep pectoral lymph nodes are noted largest measuring 9x9mm enlarged avid left axillary lymph node is noted measuring 2.6x1.3cm followed by patient underwent USG guided biopsy from the left breast lesion suggestive of IDC grade II with IHC. ER Positive 8+, PR Strong Positive 8+, Her2/Neu Negative, Ki-67 – 70%. Further, patient planned for neoadjuvant chemotherapy with AC regimen(3 weekly) x 4 cycles followed by surgery. The patient has now come for 4th cycle of neoadjuvant chemotherapy with AC regimen (3 weekly) with complaints of generalized weakness. Appetite is reduced. Bowel and bladder habits are normal. No history of fever, cough or breathlessness.

Comorbidities: Nil

Family History: Nil

Past Surgical History: Nil

Any history of missed treatment and lost for follow up: N/A



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Physical examination: PS: 1

General Condition: Moderately built and nourished.

Vital Signs: Stable. No pallor, cyanosis, clubbing, lymph nodes or pedal edema.

CVS: S1 and S2 heard. RS: NVBS+. PA: Soft, bowel sound+.

Investigations done: Blood counts and biochemistry (Outside): Report Enclosed

On 02.06.2026: Repeat CBC (HCG): Report Enclose, WNL

Treatment done: The patient received 4th cycle of neoadjuvant chemotherapy with AC regimen (INJ. ADRIAMYCIN (D1) + INJ. CYCLOPHOSPHAMIDE (D1) (3 weekly) (dose reduced) + INJ. APRECAP 150 mg along with adequate IV hydration and supportive care.

INJ. PEGASTA 6 MG SUBCUTANEOUS OD ON 03.06.2026

Course in the Hospital: The patient tolerated the treatment well with no signs of acute toxicity. General condition is stable at the time of discharge.

Advice on Discharge Medication and others:

TAB. PAN 40 MG	1-0-1 X 10 DAYS (BEFORE FOOD)
TAB. PERINORM 10 MG	1-1-1 X 3 DAYS (BEFORE FOOD)
TAB. EMESET 4 MG	1-0-1 X 3 DAYS (BEFORE FOOD)
TAB. OLANZAPINE 2.5 MG	1-0-1 X 10 DAYS (BEFORE FOOD)
TAB. ULTRACET	1-0-1 FOR PAIN (AFTER FOOD)
TAB. SHELICAL OS	0-1-0 X DAILY (AFTER FOOD)

FOR CONSTIPATION:

SYP. LOOZ 30 ML AT NIGHT FOR CONSTIPATION WHEN NEEDED

FOR GASTRITIS:

SYP. MUCAINE GEL 2 TSP THREE TIMES A DAY (BEFORE FOOD)

ORAL HYGEINE PROPHYLAXIS:

MOUTHWASH THREE TIMES A DAY



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Follow up

- To do blood test (CBC, LFT, Creatinine, Sodium, Potassium) and 2D Echo on 22.06.2026
- To review in OPD with reports on 23.06.2026 (plan to assess for surgery)

Doctors:

Dictated By: Dr. Sunita Jain	Electronically signed By:
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Dr. Shekar Patil / Dr. Shashidhara H P / Dr. Ravi Chippeswamy
9341245961 / 9341234349 / 8197710826

Mr. Venugopal A N
Nurse Practitioner
9845480217

Mr. Melbin Sebastian
Nurse Practitioner
9916317748

Mrs. Pavana M
Nurse Practitioner
9008109681

Mr. Rudramuni T
Nursing Practitioner
9611751631

Mr. Ramesha.B.H.
Nurse Practitioner
8197854443

Mr. Lakshmipathi Devareddy
Sr. Executive Operations
9008261233

Discharge Summary has been explained & handed over by: _____

Discharge Summary has been understood & received by: _____

Please call up & Fix appointments for your review/revisits.
For Appointments and Enquiry contact: 080 4020 6000 / 6358888801
For Admission contact: 080- 61562224
For Centralized customer care: 7406499999
For any Lab pending reports, contact report dispatch counter at Tower 3 Ground Floor between 09:30 am to 05:00 pm on working days (Monday to Saturday) or contact no: 080-40206106 / 6107.



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HCG Offers Home Healthcare Services:
Nursing Services, Doctor Consultation, Lab services, Rehabilitation Services, Pharmacy Delivery.
Home healthcare services contact Number: 080 -6156 6351 / 9632566011
E-mail ID: homehealthservices@hcgel.comS

Signs to watch out for and when to call emergency

In case: Fever ($>100^{\circ}\text{F}$ [Fahrenheit]), Cough, Throat pain, breathing difficulty, mouth ulcer, bleeding from any site, loose stools, abdomen pain, vomiting - Visit HCG Hospital Emergency Department Tower 3 Ground Floor
Emergency Department Contact No: 08061562265

ತುರ್ತುಸ್ಥಿತಿಗೆ ಕರೆ ಮಾಡಲು ಮತ್ತು ಯಾವಾಗ ಕರೆ ಮಾಡಬೇಕು ಎಂಬುದನ್ನು ಗಮನಿಸಬೇಕಾದ
ಚಿಹ್ನೆಗಳು:

ಒಂದು ವೇಳೆ: ಜ್ವರ ($>100^{\circ}\text{F}$ ಫ್ಯಾರನ್ಹೀಟ್), ಕೆಮ್ಮು, ಗಂಟಲು ನೋವು, ಉಸಿರಾಟದ ತೊಂದರೆ, ಬಾಯಿ
ಹುಣ್ಣು, ಯಾವುದೇ ಸ್ಥಳದಿಂದ ರಕ್ತಸ್ರಾವ, ಭೇದಿ, ಹೊಟ್ಟೆ ನೋವು, ವಾಂತಿ ಗಮನಿಸಿದರೆ- ಎಚ್ ಸಿ ಜಿ ಆಸ್ಪತ್ರೆ
ತುರ್ತು ವಿಭಾಗ ಟವರ್ 3 ನೆಲ ಮಹಡಿಗೇ ಭೇಟಿ ನೀಡಿ.

ತುರ್ತು ವಿಭಾಗ ಸಂಪರ್ಕ ಸಂಖ್ಯೆ: 08061562265

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